Machine Translated by Google

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

PRESIDENTIAL EBOLA TASK FORCE 17

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°176/MVE-BDBV/06/11/2026

Reporting date: November 6, 2026 Publication date: November 7, 2026

HIGHLIGHTS (last 24 hours)

Over the past 24 hours, seventy-six (76) new confirmed cases and 35 confirmed deaths, including 22 community deaths and 13 deaths within treatment centers, have been reported. The new cases were recorded in Ituri (47), North Kivu (24), Haut-Uélé (4), and North Ubangi (1). A total of 456 new samples were analyzed in the laboratories that submitted their data; 76 of these samples tested positive for the Bundibugyo virus, representing a positivity rate of 16.7%. No new health zones were affected in the past 24 hours.

ACCUMULATIVE CASE	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
10879	5302	48.7%	1016	2,935	75.4%

Affected provinces and affected health zones

8 Provinces affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo, Bas-Uélé, South Ubangi, and North Ubangi

67 out of 178 health zones affected (37.6%)

Summary of key points from the last 24 hours

On November 6, 2026, seventy-six (76) new confirmed cases of Ebola virus disease (EVD) were reported in four provinces: Ituri (47 cases), North Kivu (24 cases), Haut-Uélé (4 cases), and Nord-Ubangi (1 case), representing a 22.6% increase compared to the previous day (62 cases). Among these cases, 22 community deaths were reported (12 in Ituri and 10 in North Kivu). In addition, 13 confirmed deaths were recorded in Ebola Treatment Centers (7 in Ituri, 5 in North Kivu, and 1 in Tshopo), bringing the total number of confirmed deaths reported during this period to 35.

In addition, 23 patients (7 in North Kivu and 16 in Ituri) were declared cured after obtaining two negative results in the control test for Ebola virus disease.

Since the start of the epidemic, the DRC has recorded 10,879 confirmed cases, including 5,302 deaths, representing an overall case fatality rate of 48.7%. Ituri remains the epicenter of the epidemic, accounting for 69.8% of cumulative confirmed cases and 61.8% of new reported cases, while North Kivu represents 31.6% of new cases and has a case fatality rate of 54.7%, higher than the national average.

No new health zones have been affected in the last 24 hours.

Thus, 67 out of 178 health zones (37.6%), spread across eight provinces, have been affected since the start of the epidemic, namely: Ituri, North Kivu, Haut-Uélé, Tshopo, South Kivu, Bas-Uélé, South Ubangi and North Ubangi.

Machine Translated by Google

Table 1. Distribution of confirmed cases and deaths by affected province, as of November 6, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	47	7590	3584	47.2%	28/36 (77.8%)
North Kivu	24	2685	1469	54.7%	17/34 (50.0%)
Haut-Uélé	4	481	211	43.9%	7/13 (53.8%)
Tshopo	0	96	31	32.3%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	13	4	30.8%	4/11 (36.4%)
South Ubangi	0	2	1	50.0%	1/16 (6.3%)
North Ubangi	1	9	1	11.1%	2/11 (18.2%)
Total	**76**	**10879**	**5302**	**48.7%**	**67/178 (37.6%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of November 6, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed death
Ituri	**7590**	**3584**	**47.2%**	**47**	**12**	**7**	**19**
Adja	11	1	9.1%				0
Aru	25	7	28.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	220	50	22.7%				0
Boga	2	2	100.0%				0
Bunia	2038	717	35.2%	7	2		2
Damascus	41	10	24.4%				0
Drodro	15	7	46.7%				0
Fataki	84	31	36.9%				0
Gethy	11	7	63.6%				0
Kambala	2	0	0.0%				0
Kilo	40	13	32.5%				0
Komanda	193	119	61.7%				0
Lita	335	168	50.1%	1	0		0
Logo	14	9	64.3%				0
Lolwa	37	11	29.7%				0
Mahagi	6	1	16.7%				0
Mambasa	131	67	51.1%	5	0	1	1

Machine Translated by Google

Mandima	542	290	53.5%	16	6		6
Mangala	414	177	42.8%	5	0		0
Mongbwalu	731	358	49.0%	3	1	2	3
Nia-Nia	344	201	58.4%	1	0	1	1
Nizi	862	317	36.8%	3	1		1
Nyankunde	141	45	31.9%			1	1
Rimba	29	7	24.1%	1	0		0
Rwampara	1216	442	36.3%	5	2	2	4
Tchomia	86	27	31.4%				0
To ventilate	N / A	493	N / A				0
North Kivu	**2685**	**1469**	**54.7%**	**24**	**10**	**5**	**15**
Blessed	809	521	64.4%	12	7	3	10
Good	27	11	40.7%	1	1	0	1
Butembo	347	203	58.5%				0
Goma	1	0	0.0%				0
Kalunguta	109	44	40.4%				0
Katwa	684	423	61.8%	1	0	0	0
Kayna	3	2	66.7%				0
Kyondo	47	22	46.8%	1	0	0	0
Lubero	43	16	37.2%				0
Mabalako	124	40	32.3%				0
Alimbongo	6	4	66.7%				0
Manguredjipa	16	5	31.3%				0
Masereka	48	15	31.3%	0	0	1	1
Musienene	164	81	49.4%	1	1	1	2
Mutwanga	95	22	23.2%	7	0	0	0
Oicha	104	40	38.5%	1	1	0	1
Vuhovi	58	20	34.5%				0
Upper Uele	**481**	**211**	**43.9%**	**4**	**0**	**0**	**0**
Boma Mangbetu	46	20	43.5%				0
Dungu	1	1	100.0%				0
Gombari	3	1	33.3%				0
Isiro	121	56	46.3%	1	0	0	0
Pawa	122	67	54.9%	2	0	0	0
Rungu	9	5	55.6%				0
Wamba	179	61	34.1%	1	0	0	0
Tshopo	**96**	**31**	**32.3%**	0	0	1	1
Bafwasende	18	3	16.7%				0
Kabondo	24	5	20.8%				0
Lubunga	1	0	0.0%				0
Makiso - Kisangani	37	15	40.5%			1	1

Machine Translated by Google

Mangobo	10	7	70.0%				0
Tshopo	5	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	**3**	**1**	**33.3%**	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	**13**	**4**	**30.8%**	0	0	0	0
Buta	1	1	100.0%				0
Viadana	6	1	16.7%				0
Dakwa	1	0	0.0%				0
Ganga	5	2	40.0%				0
South Ubangi	**2**	**1**	**50.0%**	0	0	0	0
Bulu	2	1	50.0%				0
North Ubangi	**9**	**1**	**11.1%**	1	0	0	0
Wapinda	5	1	20.0%				0
Yakoma	4	0	0.0%	1	0	0	0
Total	**10879**	**5302**	**48.7%**	**76**	**22**	**13**	**35**

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA: Not Applicable.

Table 3. Status of alerts notified by province for November 6, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects No investigated	Number of suspects investigated and transferred At CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased			
Haut-Uélé	119	5	124	18	5	101	0	23	13	10
Bas Uélé	111	3	114	8	3	108	0	11	0	11
Ituri	1,466	72	1538	111	68	1345	4	179	99	179
North Kivu	2,174	84	2258	197	83	1667	0	280	149	280
South Ubangi	18	0	18	8	0	5	0	8	1	8
North Ubangi	ND 124	ND	ND	ND	ND	ND	ND	ND	ND	ND
Tshopo		0	124	5	0	119	0	5	0	5
Total General	**4012**	**164**	**4176**	**347**	**159**	**3345**	**4**	**506**	**262**	**493**

Machine Translated by Google

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

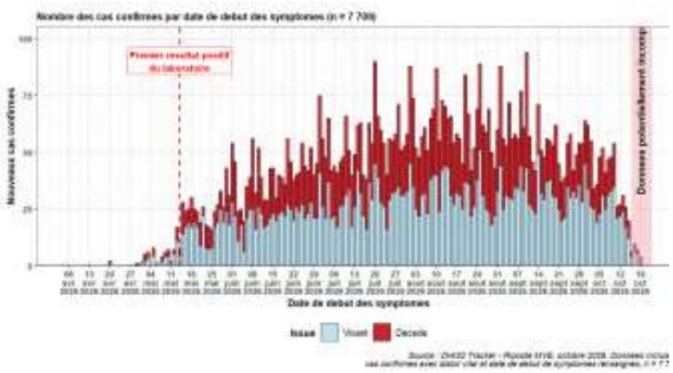
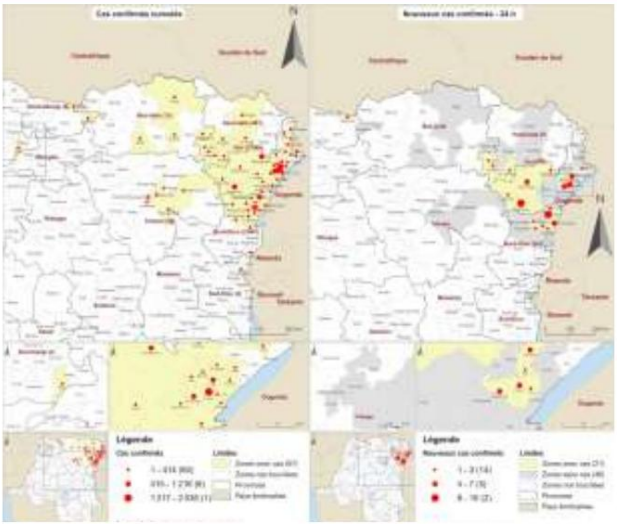


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

• Support for response activities in the provinces

• 2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.2. Main Actions

As of November 6, 2026, 4,176 alerts had been reported in the six provinces that submitted their data (Bas-Uélé, Haut-Uélé, Ituri, Nord-Kivu, Sud-Ubangi, and Tshopo), including 4,012 alerts of living persons and 164 of deaths; 3,855 were verified (92.3%). Of these, 506 (13.1%) were validated as suspected cases and 3,349 (86.9%) were invalidated. Of the 506 validated suspected cases, 506 were investigated (100%) and 493 were transferred to COVID-19 treatment centers (Table 3).

As of November 6, 2026, a total of 33,053 contacts were under follow-up in the five provinces that reported contacts to follow up, of which 24,930 were seen in the last 24 hours, representing a follow-up rate of 75.4%, below the required threshold of 95%.

Table 4. Status of contact tracing by province, as of November 6, 2026

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri	11,168	8,956	80.2%
Haut-Uélé	2,113	1,618	76.6%
North Kivu	18,444	13,546	73.4%
Tshopo	1,076	559	52.0%
Bas-Uélé	252	251	99.6%
South Ubangi	ND	ND	ND

Machine Translated by Google

North Ubangi	0	0	N / A
Total	33,053	24,930	75.4%

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

For November 6, 2026, four (4) alerts were notified, two (2) in Ituri and two (2) in North Kivu, with none reported in South Kivu, Tshopo, or Bas-Uélé (Table 5). In Ituri, the two live alerts were validated and transferred. In North Kivu, at the Kasitu PoC (Beni), one live alert was validated and referred to the Beni General Referral Hospital (100%; 1/1), and the deceased body from Lukaya (Ituri) was intercepted, validated, and sent to the morgue for swab (100%; 1/1). In total, the 3 validated live alerts were transferred to CT/CTE (100%; 3/3), the intercepted body was swabed or secured (100%; 1/1) and the 4 alerts were investigated within 24 hours (100%; 4/4).

Table 5. Status of key PoE/PoC indicators by province, as of November 6, 2026

Indicators	Ituri	North-Kivu	South Kivu	Tshopo	Bas-Uele	Overall
Report completeness	92% (57/62)	95% (20/21)	56% (5/9)	100% (18/18)	50% (11/22)	84.1% (111/132)
Strategic PoC/PoE	1	N / A	N / A	N / A	N / A	1
Number of past travelers	147 103	95 793	7 173	23,989	2,345	276,403
Proportion of travelers screened	97%	99%	98%	100%	98%	98%
% of travelers who washed their hands	96%	99%	98%	99%	98%	97%
% of those travelers made aware	100%	99%	100%	100%	76%	99%
Number of alerts notified	02	02	00	00	00	4
Number of validated live alerts	02	01	00	00	00	3
Number of lifeless bodies intercepted	00	01	00	00	00	1
% of suspected cases transferred to CT/CTE	100% (2/2)	100% (1/1)	0%	0%	0%	100% (3/3)
% of lifeless bodies swabed/secured	0%	100% (1/1)	0%	0%	0%	100% (1/1)
% of suspected cases investigated within 24 hours	100% (2/2)	100% (2/2)	0%	0%	0%	100% (4/4)

2.2.2. Challenges

In Bas-Uélé, the completeness rate is only 50% (11/22): 5 Points of Contact (PoCs) in the Viadana Health Zone did not function due to a strike by service providers, and 6 did not report due to a lack of connectivity and mobility. Only 76% of travelers were informed, and coverage remains at 22 out of 61 sites (36%). **In Ituri,** the completeness rate is 92% (57/62), with 4,906 travelers (3.3%) refusing screening, with high refusal rates in Nzebi (49%) and Pluto.
(32%), Poipo (25%) and Frigo (24%), three PoCs in the Nia-Nia health zone did not function (claim for two months of payment), the Avakubi PoC remains closed due to insecurity and the Bella PoC (Mandima) did not transmit its data; **in Haut-Uélé,** only 9 out of 12 PoCs reported, 620 travelers (3.9%) refused screening, including 440 in Bangwene (36%), and the Magambe PoC remains closed due to the service provider strike; in Sud-Ubangi, the Libenge and Dongo posts did not transmit their reports (completeness: 60%; 3/5)

North Kivu: During the night of Thursday 5 to Friday 6 November 2026, an ADF attack in the Baswagha-Madiwe group (Beni territory) led, according to a health source, to the escape of several patients, including high-risk contacts of Ebola virus disease, from the center where they were under observation.

Machine Translated by Google

Laboratory

2.2.3. Main actions

- **Ituri:** 47 new positive results (36 living and 11 dead) out of 327 samples received and analyzed (positivity: 14.3%).

North **Kivu:** 14 new confirmed cases (4 living and 10 dead); laboratories in Beni and Butembo have 302 samples were received and 298 were analyzed (positivity: 4.7%)
Tshopo: 0 new positive results out of 6 samples received, including 4 live and 2 dead

- Haut-Uélé: 4 new positive results (4 live) out of 29 samples analyzed at the Isiro laboratory (positivity: 13.8%)
- **Bas Uélé:** 28 samples (28 blood and 0 swab) from Bondo, Buta and Titule were analyzed at the laboratory of Buta and 15 others from Ganga and Viadana at the Isiro laboratory, all with negative results.

2.3. Infection Prevention and Control (IPC/EDS)

2.3.1. Main actions

- **North Kivu:** 17 out of 20 rings open with a significant gap in the Beni health zone, 8 households decontaminated out of the 26 households identified with low coverage in Katwa, Butembo and Beni

- **Tshopo:** 1 ring opened (Makiso-Kisangani) and decontamination of a household around the 94th case (Kabondo) and of a health center around the 95th case (Makiso-Kisangani), as well as of the inspection office and the main hall of the DPS; training and capacity building: completion of the training of 60 PPLs from DHKIS on IPC before its reopening, monitoring and support of 17 out of 17 health centers in 5 health zones with 216 providers briefed on standard precautions (100%), assessment of 3 high-risk PPLs from the Makiso General Referral Hospital around the 95th case and inventory of EVD transit sites in 5 health centers in Mangobo; cumulatively, 59 out of 107 health centers (55.1%) received IPC kits and 8 out of 107 (7.5%) have a score greater than 80%; 1 complete EDS made and 1 swabé body.

- **Haut-Uélé:** 4 rings opened in Pawa, 20 households decontaminated out of 20 (100%) and 6 out of 6 ESS (100%), including the Pawa CTE around confirmed PPL; training and capacity building: 11 nurses briefed in Wamba on the functionality of DLM, safe injection and standard precautions, with provision of PCI kits and functional triage in the 4 ESS visited in the ZS; 4 EDS carried out on 4 alerts (100%) and 4 bodies swabés (Pawa, Boma Mangbetu and Wamba).

2.3.2. Bas-Uélé: monitoring and support of 11 out of 11 ESSs in the health zones of Buta (7) and Ganga (4); training and capacity building: briefing of 22 out of 22 service providers and 2 out of 2 hygiene workers (100%) on the mandatory wearing of PPE, waste management, sorting operation, hand washing and preparation of chlorinated solutions; awareness-raising in public places in Buta (Rawbank, central market and FARDC market).

2.3.3. Challenges.

In Bas **-Uélé,** only 130 out of 632 service providers (20.6%) are trained in IPC-WASH, 16 out of 20 priority health facilities (HHFs) lack functional triage, 17 out of 20 lack water for 72 hours, none of the 20 have mattresses, the available chlorine is limited to 45 kg, no WASH partners support activities around cases, and the service provider strike has intensified. **In Haut-Uélé,** opening the rings remains difficult in Isiro due to community resistance, only 4 out of 7 health zones have submitted their IPC data, IPC kits are lacking for HHFs, IPC supervisors are not activated in 4 health zones (Rungu, Dungu, Boma Mangbetu, and Gombari), and the EDS teams trained by FHI 360 are not yet operational. **In Ituri,** only 15 out of 29 rings (51.7%) could be opened, teams remain insufficient in Mangala, Mandima, Damas and Mambasa, IPC inputs are lacking in Mandima, Mambasa and Nia-Nia, the strike is disrupting activities in Mangala and 16 sampling failures were recorded on bodies.

2.4. Holistic care

2.4.1. Main actions

In Ituri, 116 new admissions (38 confirmed and 78 suspected, including 6 admitted in the advanced stage) were recorded, compared to 96 discharges, including 7 recoveries and 1 discharge without medical advice in Nizi. Thus , 344 patients (116 confirmed and 228 suspected) remained in isolation at the end of the day (341 the previous day), representing an overall occupancy rate of 34%; 329 out of 336 patients (97.9%) received hot meals, all children under LNPE/SLM were cared for (8/8 at the CTE, 4/4 in daycare, and 66/66 in the community), 31 patients were evacuated, and 18 out of 34 transfers (52.9%) were organized.

In North **Kivu,** 140 new admissions (13 confirmed and 127 suspected) were recorded, compared to 146 discharges, including 12 recoveries. A total of 578 patients (192 confirmed and 386 suspected, including 65 children under 5 and 25 critically ill) remained in isolation at the end of the day (595 the previous day), including 184 outside of Ebola Treatment Centers (ETCs).

Machine Translated by Google

with CTE/CT occupied at 90.8% (394/434 beds); 11 out of 17 ambulances (64.7%) provided 19 trips, all successful (100%), for 21 calls received, 15 confirmed unstable patients benefited from specific diets, the 3 malnourished children and FEFA (100%) were taken care of and the 17 separated or orphaned non-breastfed infants are on SLM (100%).

Low coverage of SMSPS interventions around the latest confirmed cases in health zones Biena, Manguredjipa, Kayina, Lubero, Masereka , Kalunguta, Mabalako, Mutwanga, Vuhovi; Absence of nurseries in 10 affected health zones for the support of children from families;
Shortages of trained human resources in SMSPS/ZS deployed in the context of Ebola Virus Disease 17; Low coverage of providers and pillar members trained in SMSPS in the context of Ebola Virus Disease 17

2.4.2. **In Tshopo,** Confirmed Case No. 84 has refused hospitalization at the CTE for 12 days (Kabondo Health Zone, AS Gloria);
Community resistance to investigations, isolation or transfer to the CTE and contact tracing
Low increase in alerts (especially deaths)
Threats against the response team (especially in the Bafwasende, Kabondo and Mangobo health zones)
In Haut **-Uélé,** 13 new admissions (4 confirmed in Isiro and 11 suspected) were recorded, compared to 1 exit.

2.5. Vaccination

2.5.1. Main actions

In Ituri, 194 people were vaccinated during the day (105 women, including 3 pregnant and 5 breastfeeding women, and 89 men), bringing the total to 3,023 people vaccinated (including 31 pregnant women and 89 breastfeeding women), with 41 non-severe and 5 severe AEFI reported since the beginning; vaccination of people living with HIV and research on the effectiveness of Ervebo against BDBV Ebola virus disease are continuing. In **North Kivu,** 299 people living with HIV were vaccinated (149 at the Kasabinyole Health Center, Beni Health Zone, and 150 at the Kalunguta General Referral Hospital), bringing the total to 2,266.
vaccinated (1,282 men and 984 women, including 16 breastfeeding women) in 4 health zones (Beni: 1,790; Oicha: 200; Kalunguta: 150; Mabalako: 126) out of 7,105 expected (31.9%), with 334 doses remaining out of 2,600 received, no doses lost and 1 non-serious AEFI reported.

• **TSHOPO:** 13,036 people vaccinated in the six health zones, representing 87.1% coverage with 924 remaining as used.
Vaccination has vaccinate there target Currently coverage. Three health zones exceed 100% of their target: Bafwasende (120.0%), Makiso-Kisangani (109.3%) and Tshopo (103.8%), while Kabondo records the lowest coverage (43.0%).
Vaccination strategy: Vaccination is currently being implemented on a compassionate, rather than preventive, basis, requiring clarification of guidelines regarding eligibility and the initiation of vaccination for contacts. Data quality: A discrepancy of 544 vaccinated individuals exists between Excel (13,036) and DHIS2 (12,492), primarily in the health zones of Tshopo (259) and Mangobo (80).
Makiso-Kisangani (101)

In Haut **-Uélé,** 3,000 doses of Ervebo were received at the Isiro Expanded Program on Immunization (EPI) center, and members of parliament and army officers were briefed at the Isiro General Referral Hospital (HGR) vaccination site. **In Bas-Uélé,** the compassionate vaccination support team remains deployed in the Bili health zone while planning is underway for Bondo and Monga. Field workers have been paid, and the vaccination of motorcycle taxi drivers is being prepared. Cumulatively, 2,000 out of 13,202 targeted individuals (15.1%) have been vaccinated in the province. **In Nord-Ubangi,** 500 doses of Ervebo were received for the compassionate vaccination of people living with HIV (PLHIV) and people living with HIV (PLHIV), scheduled for November 9-14, 2026, following briefings in the eight targeted health zones.

2.5.2. Challenges

In Ituri, the TPL/PPL target remains to be redefined and 5 serious AFIs have been reported cumulatively; in North Kivu, vaccination remains concentrated in the Beni health zone (79.0% of those vaccinated), coverage reaches only 31.9% of the 7,105 people expected and there are only 334 doses left, or 12.8% of the stock received, hence the request for supplies addressed to the national coordination; in Bas-Uélé, the start-up of the CCUs is still not effective and the Bili health zone has not yet had any vaccinated individuals (0/1,067); in North Ubangi, vaccines still need to be made available in sufficient quantities.

Machine Translated by Google

2.6. Continuity of Care

2.6.1. Main actions • In North Kivu,

the assessment of the continuity of essential health services at the Mambowa General Referral Hospital (Biena) revealed the closure of the emergency department without any alternative, a non-functional 24-hour triage, weak protection of staff and 18 agents in quarantine without measures to maintain services, with only 11% of staff trained in IPC.

• **In Tshopo,** 3 ESS were evaluated (CM Béthanie and Clinique Familia, ZS of Tshopo, and CS Unikis of the Central Campus, ZS of Makiso-Kisangani): oxytocin is available in all three structures, but triage is not effective, none has an incinerator, the Central Campus has neither a garbage pit nor reception, consultation and laboratory registers, only 1 out of 13 providers is trained on Ebola at Clinique Familia (7.7%) and none at the Central Campus or at Béthanie, staff vaccination remains low (18% at the Central Campus, 23% at Familia and 38% at Béthanie) and, at Béthanie, 5 out of 20 children (25%) missed their CPS appointment and 8 out of 15 pregnant women (53%) missed their ANC appointment; The recovery plans for CM Mbuyu Butoke II and CS Lobiko have been developed and the training of managers and service providers on transfusion safety has continued (J3).

• **In Haut-Uélé,** the integrated supervision of the mosquito net distribution campaign continued in the health zones of Niangara and Dungu with the integration of Ebola messages and routine vaccination, and work on the recovery plan for the Ebola treatment centers of the Provincial Health Division began; in Bas-Uélé, the strike by nursing providers is affecting the entire province.

2.6.2. Challenges

Implementation of the recovery plans for social and solidarity economy (SSE) assessed in Tshopo is still pending, where the strike by public SSE staff and the non-activation of the Expanded Program on Immunization (EPI) continuity plan persist, with a decrease in the use of curative services, IMCI, ANC, maternity and vaccination services; in North Kivu, the closure of the emergency department at Mambowa General Referral Hospital and the quarantine of 18 staff members without a service maintenance plan compromise access to care in Biena; in Haut-Uélé, the population deplores the insufficient quality of care offered in health facilities; in Bas-Uélé, the indefinite strike by nursing providers paralyzes the provision of care throughout the province; no data transmitted by Ituri, Sud-Ubangi and Nord-Ubangi.

2.7. Risk Communication and Community Engagement (RCCE)

2.7.1. Main actions

• **In Ituri,** 16,798 home visits reached 109,472 people, 275 educational talks reached 4,236 people, 13 community debates reached 288 people, 17 mass awareness campaigns reached 2,302 people and 6 advocacy campaigns reached 29 authorities, including religious authorities; 455 community alerts, including 209 deaths, were notified, 42 out of 47 feedback reports (89.4%) and 4 out of 7 incidents (57.1%) were resolved within 48 hours; 129 motorcycle taxi drivers from the Ndjili parking lot (Bunia) and 123 students from the Magagi Primary School (Lembabo School Association) were sensitized and the release of 3 recoveries was celebrated in Mambasa; In support of the other pillars, the CREC facilitated the pre-listing of 1,258 contacts around 9 confirmed cases, the decontamination of 26 out of 26 households (100%), including 1 structure and 1 church, and 5 ESS, the completion of 40 out of 43 EDS (93.0%) and the securing of 11 bodies, the announcement of 21 positive results out of 22 and the transfer of 55 out of 61 suspects (90.2%) to CTEs; only 11 out of 28 ZS (39.3%) reported.

• **In North Kivu,** 34,748 home visits reached 119,978 people, 169 educational talks reached 7,350 people, 85 mass communication sessions reached 6,223 people and 6 briefings reached 533 people; 582 out of 650 expected alerts (89.5%) were notified, 23 out of 34 feedbacks (67.6%) and 1 out of 7 incidents (14.3%) were resolved within 48 hours and 346 out of 4,750 people (7.3%) were engaged in community actions; in Beni, the 2nd day of updating the CREC plan was devoted to cross-functional work; In Butembo, following a plea, the pastor in charge of the Pilgrims Without Borders church pledged to deny his messages against prevention measures, and the release of 4 recovered and 5 non-cases was accompanied by a media testimony on the importance of early transfer to CT/CTE; 300 students from the Buyori Institute (Mutwanga) and more than 367 schoolchildren and 15 teachers from the Mangodomu Primary School (Mabalako) were sensitized, and the CREC facilitated the listing of 56 contacts and the decontamination of 4 households and 2 structures around the latest cases in Butembo, Mabalako, Vuhovi and Katwa.

In Haut-Uélé, 322 home visits reached 2,254 people, 10 educational talks reached 110 people, and 9 mass activities reached 2,225 people, for a total of 4,589 people reached; 8 community alerts were reported (6 in Wamba and 2 in Isiro) ; a meeting with community health workers and community leaders, chaired by the deputy mayor of Isiro, focused on intensifying awareness campaigns in markets and transport agencies; dialogue was facilitated with young people suspected of causing unrest in Bolebole, who pledged to support the response; and 12 people linked to 2 suspected community cases were sensitized to contact tracing, decontamination, and EDS in the Gossamu health district; the CREC also provided support

Machine Translated by Google

decontamination of CH Mayogo and 2 households and supervised the integration of EVD messages into mosquito net distribution.

- **In Tshopo,** 210 home visits reached 3,645 people, 4 educational talks reached 421 people and 10 mass awareness campaigns reached 5,508 people (Lubunga, Kabondo, Makiso, Bafwasende and Tshopo); 48 community alerts were notified (26 in Makiso, 18 in Tshopo and 4 in Kabondo) and 11 feedbacks out of 11 (100%) were resolved within 48 hours; 25 RECOs from the Makiso-Kisangani Health Zone were trained with the support of the project "Acting today for a future without an Ebola epidemic", the mapping of CREC's PTFs was updated and a handwashing demonstration was organized in Kabondo schools in support of IPC.

In Bas **-Uélé,** 486 home visits reached 2,501 people, one educational talk reached 23 people, two mass awareness campaigns reached 435 people, and two radio programs reached approximately 25,985 listeners in Buta. Twenty-three students from the Dingila Technical Institute (ITM) were briefed on Ebola Virus Disease (EVD) and community engagement, and an awareness campaign was conducted at the opening of the academic year at the Free Protestant University in Africa. No community alerts were reported, and only 4 out of 13 health zones (ZS) submitted reports. In Nord-Ubangi, the CREC subcommittee met, and the dissemination of awareness messages continues on local radio stations.

2.8. Mental Health and Psychosocial Support (MHPSS)

2.8.1. Main actions

In Ituri, 25 of the 30 confirmed cases (83.3%), 68 confirmed cases out of 91 under follow-up, 43 new suspected cases out of 96, and 126 suspected cases out of 132 under follow-up have been managed; 16 out of 19 (84.2%) households with separated and orphaned children have received support; 88 children are being monitored, including 22 out of 22 in daycare with 4 new admissions and 1 family reunification; 5 out of 7 have recovered and been reintegrated, and 28 out of 33 are being monitored in the community; and 95 results have been announced, including 25 positive results; 247 caregivers and 77 people living with HIV have received support, and the capacities of Psychologists and APS/TPS in Rimba, Logo and Mahagi have been reinforced.

- **In North Kivu,** 8 out of 9 notifying health zones were covered, 42 of the last 52 confirmed cases (80.8%), 216 confirmed cases under follow-up, 116 new suspected cases and 165 suspected cases under follow-up were taken care of, 32 out of 41 (78.0%) separated and orphaned child households were supported, 5 out of 5 children were followed up in daycare and 28 out of 36 (77.8%) in the community, 16 out of 16 followed up in the community were cured, 104 results were announced, including 52 positive results, 549 people (183 men, 267 women and 99 children) benefited from psychoeducation (UNICEF support), 89 households were supported for public health actions and 68 PPLs were supported; Training for psychologists, community health workers, and community mobilizers on SMSPS support for recoveries was provided in Oicha (Eringeti health district). In Haut-Uélé, no SMSPS data was transmitted for the day.

In Tshopo, 6 out of 7 reporting health zones were covered, all 5 confirmed cases under observation at the Meteorological Treatment Center (5/5) received psychological support, the new suspected case of the day and the 3 suspected cases under observation were treated (100%), 1 negative result was announced, 33 caregivers and 1 person receiving psychoeducation on barrier gestures, 2 households received support, the removal of resistance at the Rekapi II morgue continues, and the recovered patient being monitored in the community was seen (1/1). In Bas-Uélé, no SMSPS interventions were reported for the day. In Nord-Ubangi, the 12 patients isolated at the Wapinda General Referral Hospital are receiving psychological support.

2.8.2. Challenges

2.9. In Ituri, only 15 out of 28 affected health zones (53.6%) are covered, 5 of the last 30 confirmed cases (16.7%) and 23 of the 91 confirmed cases currently being monitored have not received care, and only 43 out of 96 new suspected cases (44.8%) have received support. In North Kivu, 10 of the last 52 cases (19.2%) have not received support, coverage remains low around the last cases in Biena, Manguredjipa, Kayna, Lubero, Masereka, Kalunguta, Mabalako, Mutwanga, and Vuhovi, 10 affected health zones do not have a daycare center, and 9 out of 41 households with separated or orphaned children remain without support. In Haut-Uélé, there are no daycare centers in Isiro, Pawa, Wamba, or Boma Mangbetu, and community health workers lack transportation and nutritional kits for weaned infants. In Tshopo, community health workers in the Wanie-Rukula health zone are not trained, and the local health center lacks communication equipment and motorcycles. In Bas-Uélé, psychosocial support for the three confirmed cases is still lagging.

2.10. Logistics

2.10.1. Main actions

- **In Ituri,** 31 patients were evacuated and 18 out of 34 transfers (52.9%) were organized by ambulances, but reporting of ambulance movements remains low; a medical evacuation (MEDEVAC) of a confirmed PPL from Butembo was carried out by private jet from Bunia airport to the United Kingdom; for health products and medicines (HPM), the blood shortage persists and IPC medicines and inputs are lacking in Mandima and Mambasa, where teams are also waiting for motorcycles, vehicles, fuel and an ambulance to serve the checkpoints.

Machine Translated by Google

• **In North Kivu,** 11 out of 17 ambulances (64.7%) were mobilized for 19 trips, all successful, out of 21 calls received (response capacity: 61%); supplies: handwashing kits delivered to the Mangodomu Primary School (Mabalako); for the PSM, 2,600 doses of Ervebo have been received in total (2,266 used, 334 remaining, none lost) and a new supply of vaccines has been requested from the national coordination; the construction of huts and the passenger walkway at Beni airport is planned with the support of the IOM.

• **In Tshopo,** the Cinquantenaire site was prepared for PEP (erection of tents), a room at the Makiso General Referral Hospital was prepared for the isolation of service providers exposed during post-mortem cesarean section, the delivery of materials began for the rehabilitation of the Makiso isolation and the extension of the standardized CTE of Météo, and the Kongakonga isolation site was supplied with medical inputs after updating stocks; the mobility of pillars and the transport of participants to the intra-action review were ensured and the 3 available ambulances were mobilized (100%); for the PSM, the laboratory is facing a reagent shortage (MVE buffer), the breakdown of two modules of the Radione platform and a near-shortage of diesel (1,000 liters needed for the incinerator), and the mobility need of the ZS remains estimated at 115 motorcycles.

• **In Haut-Uélé,** a new Toyota Land Cruiser vehicle was provided by the central government, through the INSP, for the mobility of the response, the ambulance of the ZS of Isiro was repaired and 7 ambulance trips transported 7 patients; for the PSM, medicines and inputs for care were delivered to the CTE of Isiro and 3,000 doses of Ervebo were received at the PEV antenna of Isiro, the fuel stock went from 988 to 928 liters and there is a lack of fuel for the generator of the ultra-low temperature cold chain, while the disappearance of hydroalcoholic gels is reported.

In Bas -Uélé, the Bondo health zone has been supplied with EDTA tubing and PCI supplies; the packaging of PCI and PEC supplies for the health zones is underway; the rehabilitation of the meeting room and offices of the Provincial Health Division (DPS) continues with the WHO; and construction of the Ebola Treatment Center (ETC) at the Buta General Referral Hospital (HGR) is 50% complete. For the Medical Support Centers (PSM), the chlorine stock is limited to 45 kg; the laboratory lacks a fire extinguisher, an uninterruptible power supply (UPS), and a computer; no diesel fuel is available; and the 20 priority health centers (ESS) lack mattresses. In Nord-Ubangi, four motorcycles have been deployed to the two affected health zones, and a shipment of WHO supplies has been received in Wapinda, but mattresses for the ETC and other supplies are still pending.

2.11. Security

2.11.1. Main actions

• **In Ituri,** the Avakubi PoC remains non-functional due to insecurity; in North Kivu, a grenade was discovered in front of the entrance to the Beni Health Zone office, EDS teams were accompanied and secured for the burial of 2 confirmed cases at the Bundji cemetery and for swabs in Butuhe and the Vighole neighborhood, the IPC team was accompanied for the decontamination of a household in Manziko (Benengule Health Area) and the threat against PoC providers on the Butembo axis persists; **in Haut-Uélé,** the security threat against PoC providers persists on the Bolebole axis (Pawa and Wamba); in Tshopo, the house of a RECO was burned down in Bafwasende, where response activities remain suspended, and young people attempted to recover a lifeless body; In Bas-Uélé, the pillar assessed the security situation, secured the response and monitored activities at the zonal level.

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)

2.12.1. Main actions

In Ituri, 2 members of the INRB CREC team (1 woman and 1 man) were briefed on the PSEAH and signed the code of good conduct, 22 wood sellers (12 women, 7 men and 3 boys) were sensitized in the Bunia health zone and the display of messages and the distribution of leaflets continue in the Bunia health zones,
Aru, Mongbwalu, Mambasa, Fataki and Mandima;

In North Kivu, 304 people were sensitized or briefed during 4 activities, including 6 focal points of
WAA briefed in the AS of Murara (Karisimbi), 285 participants sensitized in Butembo with the support of the WHO and 13 people in Beni, 146 leaflets and 4 posters distributed and the toll-free numbers 495555 and 151 disseminated;

In Haut-Uélé, 9 PPLs from the Anualité hospital in Matali (5 men and 4 women, Wamba health zone) were sensitized on the PSEA, referral mechanisms and the rights of victims; in Bas-Uélé, 23 learners from the ITM in Dingila (17 boys and 6 girls) were sensitized on the PRSEAH.

Machine Translated by Google

2.12.2. Challenges

- Coverage of the affected areas remains limited: only Ituri, North Kivu, Haut-Uélé and Bas-Uélé have reported their PSEA activities for the day (4 out of 8 DPS; 50.0%); in Bas-Uélé, 4 health zones (Viadana, Bondo, Likati, and Ango) remain silent, funding for the pillar is low, and posters and banners are out of stock; the reporting mechanism is known in only 15 out of 36 health zones (41.7%) in Ituri and 6 out of 13 health zones (46.2%) in Haut-Uélé; only one partner (WHO) reports its PSEA activities in Haut-Uélé; some partners refuse to align in Ituri and the awareness tools in French and Lingala, the visibility materials and the IT kits of the pillar remain insufficient.

KEY MESSAGES

Seventy-six (76) new confirmed cases were reported in four provinces, including 47 in Ituri, 24 in North Kivu, 4 in Haut-Uélé (Pawa 2, Isiro 1, Wamba 1) and 1 in North Ubangi (Yakoma). There transmission remains particularly marked in the ZS of Beni (12 cases) and Mutwanga (7 cases) in North Kivu and in those of Mandima (16 cases) and Bunia (7 cases) in Ituri.

Of the 35 confirmed deaths reported, 22 are deaths Community-related deaths (12 in Ituri and 10 in North Kivu) and 13 deaths at Ebola Treatment Centers (7 in Ituri, 5 in North Kivu and 1 in Tshopo). The follow-up rate Contacts were established at 75.4% (24,930/33,053) in the five provinces with contacts in follow-up (Bas-Uélé: 99.6%; Ituri: 80.2%; Haut-Uélé: 76.6%; Nord-Kivu: 73.4%; Tshopo: 52.0%, a significant decrease compared to the previous day (82.6%, or -7.2 points) and always below the 95% threshold, with the lowest rate at the Tshopo, where Bafwasende data is not available in The reason for the cessation of activities is due to insecurity.

TIMELINE OF KEY EVENTS

From April 24 to May 15, 2026, the epidemic is moving beyond the detection of the first cases suspected of having the virus confirmed by biological testing Ebola Bundibugyo, leading to the declaration official of the 17th epidemic of Virus Disease Ebola in the Democratic Republic of Congo.

Between May 17 and 18, 2026, the authorities national, with the support of partners, have strengthened the response with the declaration of the public health emergency of scope national and then continental, accompanied by

Machine Translated by Google

EBOLA PRESIDENTIAL FORCE TASK 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System

Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Email:diedonne.mwamba@insp.cd

Incident Manager

Professor Dr. AHUKA MUNDEKE Steve

Email: steve.ahuka@unikin.ac.cd

Operations Manager

Dr. KOMBE NGWAMA John

Editorial Team

Professor Dr. MULANGU KATULUMBA Félix-Médard

Dr. Jules Katusi Biselenge

Dr. SIFA

Contact:

www.insp.cd

contact@sgiebola17.org

Freephone number: 151. www.sante.gouv.cd